

HDPC

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VITAL SIGNS

Vital signs are objective measurements of the bodies most basic and essential functions.

PURPOSE OF VITAL SIGNS

1. *Assessing overall health* - they provide a quick, objective snapshot of a person's general physical health and the functioning of their basic body systems.
2. *Early detection of illness* - changes in vital signs can be the earliest indicators of an underlying health issues like an infection, cardiovascular problems or respiratory distress even before the patient shows noticeable symptoms.
3. *Diagnosis and triage* - triage acts as an emergency or urgent setting. Vital signs are used to quickly assess the severity of a patient's condition and determine which patient need immediate lifesaving care first. On the other hand, diagnosis is the narrowing down the potential cause of a patient's symptoms on the basis of abnormal vital signs, leading to a faster and more accurate diagnosis

COMPONENTS OF VITAL SIGN

- | | | |
|---------------------------|----------------------|----------------------|
| 1. Pulse rate/ heart rate | 3. Blood sugar level | 5. Oxygen saturation |
| 2. Blood pressure | 4. Body temperature | 6. Pain score |
| | | 7. Respiratory rate |

PULSE RATE

Pulse rate is the number of pulse beats in a minute. We can check the pulse manually or by using a machine.

Pulse sites -

• Carotid artery	• Radial artery	• Femoral artery	• Brachial artery
• Temporal artery	• Popliteal artery	• Posterior tibial artery	• Dorsalis pedis artery

TIPS FOR ACCURATE RESTING HEART RATE

1. *Resting* - sit quietly for at least five minutes before checking.
2. *Calm* - do not check it right after exercising, consuming caffeine or experiencing stress, as these can evaluate your rate for up to two hours
3. *Consistent* - try to check it around the same time each day. E.g. first thing in the morning before getting out of bed.

METHODS OF TESTING PULSE RATE

1. *Placement* - gently place the tips of your index and middle finger on the site.
2. *Pressure* - apply light pressure, until you feel a pulse.
3. *Count* - count the beats for 15 seconds and multiply by four

NORMAL PULSE RATE RANGE

1. **Infant** (0-12month) → **100-160** beats/min
2. **Children** (1-10years) → **70-120** beats/min
3. **Adult** (18-64 years) → **60-100** beats/min

ABNORMAL PULSE RATE

- Tachycardia - pulse rate above the normal range
- Bradycardia - pulse rate is below the normal range
- Arrhythmia - irregular pulse rhythm
- Palpitations - skipped beats or extra beats

FACTORS AFFECTING PULSE RATE

1. Physical activity
2. Emotional state
3. Body temperature
4. Medication
5. Body position

RESPIRATORY RATE

The respiratory rate is a vital sign that measures the number of breaths a person takes per minute.

METHODS OF MEASURING RESPIRATORY RATE

1. Do not inform the person that you are counting their breaths
2. Count the respiratory rate immediately after measuring the pulse
3. Observe the chest or shoulders rising and falling. one complete rise and fall is one breath.
4. Count the breaths over a full 60 seconds to check for any irregular patterns.

NORMAL RESPIRATORY RATE RANGE

Adult : **12-20** breaths/minute
 Children : **20-30** breaths/minute
 Infant : **25** breaths/minute

ABNORMAL RESPIRATORY RATE

- *Tachypnea* - rapid breathing with respiratory rate. Over 20 breaths/min in a resting adult
- *Bradypnea* - abnormal slow breathing with a respiratory rate below 12 breaths/min in a resting adult.
- *Apnea* - temporary cessation of breathing, such as during sleep apnea.

FACTORS AFFECTING RESPIRATORY RATE

1. Physical activity
2. Emotional state
3. Body temperature
4. Illness, asthma, pneumonia or heartfailure

BLOOD PRESSURE

Blood pressure is the pressure exerted by blood on the walls of blood vessels as it circulates throughout the body.

COMPONENTS OF BLOOD PRESSURE

Systolic pressure - the pressure in the arteries when the heart beats

Diastolic pressure - the pressure in the arteries when the heart relaxes

NORMAL BLOOD PRESSURE

Adult : **120/80** mm of Hg

Infant : **60-90** mm of Hg/**30-60** mm of Hg

Older adult : **150/90** mm of Hg

ABNORMAL BLOOD PRESSURE

- Hypertension - high blood pressure
- Hypotension - low blood pressure

FACTORS AFFECTING BLOOD PRESSURE

1. Age
2. Family history
3. Obesity
4. Physical activity
5. Diet (high sodium intake increases blood pressure)
6. Stress level

METHODS OF MEASURING BLOOD PRESSURE

1. Use of sphygmomanometer
2. Measure in a quiet, seated position
3. Take multiple reading at different times

OXYGEN SATURATION

Oxygen saturation measures the percentage of hemoglobin (Hb) in red blood cells (RBCs) that is saturated with oxygen.

NORMAL RANGE OF OXYGEN SATURATION

95-100%

ABNORMAL RANGES (HYPOXEMIA)

- Mild hypoxemia :90-94% (indicate a mildly low oxygen level in the blood)
- Moderate hypoxemia :80-89%
- Severe hypoxemia :below 80% (requires a high concentration oxygen mask)

METHOD OF MEASURING OXYGEN LEVEL (PULSE OXIMETER)

1. Remove any nail polish or false nails
2. Warm your hand if it is cold
3. Rest for at least 5mins before taking the measurement
4. Switch on the oximeter and place it on the middle or index finger

PURPOSE

1. To detect hypoxemia
2. To monitor respiratory status
3. To assess oxygen therapy effectiveness

BODY TEMPERATURE

Body temperature is a vital sign that indicates the body's thermal state

NORMAL RANGE OF BODY TEMPERATURE

35.5 °C - 37.5 °C

97.7 °F - 99.5 °F

ABNORMAL BODY TEMPERATURE

- Hyperthermia - body temperature higher than normal range
- Hypothermia - body temperature lower than normal range

TYPES OF FEVER

1. **Continuous fever** - temperature remains above normal i.e. more than 39 °C
2. **Remittent fever** - fluctuation in the body temperature throughout the day but it never returns to normal
3. **Intermittent fever** - temperature alternates between fever and normal levels.
4. **Relapsing fever** - fever returns after a short period (1-2days) or normal temperature
5. **Biphasic fever** - it is a fever that occurs in two distinct phases. It is often called a saddleback fever.

COMMON CAUSES OF FEVER

1. Infection (viral fever, fungal fever, bacterial fever)
2. Inflammation (fractures, inflammatory diseases)
3. Cancer fever
4. Medications/ vaccinations
5. Injection

MEASUREMENT SITES

1. Oral
2. Axillary (armpit)
3. Tympanic (ear)
4. Temporal (forehead)
5. Rectal

PYREXIA

- Pyrexia is the medical term for a fever or abnormal elevation of body temperature above the normal range
- Pyrexia is an ancient Greek word which means 'fire' and 'heat'.

PHASES OF FEVER

1. *Onset phase (cold or chill phase)* - this is the beginning stage of fever. Body temperature starts to rise. Person may feel cold, shivering, goosebumps, body aches.
2. *Fastigium phase (stadium or fever phase)* - this is the peak stage of fever, when body temperature is at its highest and remains steady for sometimes. Person may feel hot, flushed, weak & thirsty.
3. *Defervescence phase (flush or crisis phase)* - this is the phase where the fever begins to decrease and the body temperature returns to normal. Person may sweat a lot, feel tired, or weak as the power breaks.

BLOOD SUGAR LEVEL

Blood sugar level or blood glucose level is the amount of glucose present in the blood.

TYPES OF BLOOD SUGAR LEVEL

1. Fasting blood sugar (FBS) - sugar level before food
2. Postprandial blood sugar (PPBS) - sugar level 2 hours after food
3. Random blood sugar (RBS)

ABNORMAL RANGE (HYPERGLYCEMIA)

Hyperglycemia means high blood sugar. It occurs when the amount of glucose in the blood is higher than the normal range, usually, above 100mg/dl fasting or above 140 mg/dl after meals. In case of RBS, it is greater than 120.

TYPES OF HYPERGLYCEMIA

Hyperglycemia is of two types -

1. Pre-diabetes -
 - a. FBS : **100-125** mg/dl
 - b. PPBS : **140-199** mg/dl
2. Diabetes -
 - a. FBS : ≥ 126 mg/dl
 - b. PPBS : ≥ 200 mg/dl
 - c. RBS : ≥ 200 mg/dl

METHODS FOR MEASURING BLOOD SUGAR LEVEL

1. Using a glucometer
2. Laboratory blood test

PAIN SCORE

Pain score is a clinical tool used in healthcare to systematically measure and monitor a patient's subjective experience of pain

TYPES OF PAIN SCORE

1. Numeric rating scale - for children (9-10 years) and adults
2. Usually analogue scale
3. Wong-baker faces pain scale

INSTRUCTIONS FOR TAKING VITAL SIGNS

1. Patient's position
2. Choose the right site
3. Use proper techniques
4. Documentation

RECORDS AND REPORTSRECORDS

Records are the written document maintained by medical practitioner/institution that provides a permanent, continuous and systematic account of medical history, examination, treatment and findings of a patient.

They serve as evidence of what a doctor observes, did decided at each step

REPORTS

Reports are the formal written statements based on medical reports, prepared by the doctor on request of legal authorities (police, court, insurance, government). They summarized medical findings and opinions related to a specific medico legal question

e.g. medical legal injury report, post mortem reports, death certificate, disability certificate, cause of death certificate

TYPES OF RECORDS

Medical records are maintained in hospital and clinic for both medico legal and non-medico legal purposes.

Clinical records -	Medico-legal records	Administrative records
a. OPD records b. IPD care sheets c. Nursing notes d. Treatment charts e. Consent form f. Discharge summary g. Laboratory, imaging reports	a. Medico-legal register b. Injury reports c. Sexual assault report d. Age estimation records e. Post-mortem register f. Death certificate g. Toxicology sample recording	a. Admission register b. Staff duty records c. Inventory & drug records

TYPES OF REPORTS

1. Medical reports - e.g. clinical summary, case reports, treatment report
2. Medico-legal report - e.g. injury report, sexual assault examination report, post mortem examination report
3. Biscera converting report
4. Disability assessment report
5. Poisoning certificate

IMPORTANCE OF RECORDS

Medical records have huge, legal, academic and administrative value

1. *Legal value - it acts as a primary evidence in court*
 - a. Protects doctor from false accusation
 - b. Help in delivering justice
 - c. Useful for insurance care
 - d. Essential for police investigation
2. *Medical values*
 - a. Continuity of patient care
 - b. It helps feature diagnosis and treatment
 - c. Basis for reference and follow up
3. *Administrative value*
 - a. Hospital management audits
 - b. Statistical data for planning
 - c. Quality control and accreditation (ISO,NABH)
4. *Research & educational value*
 - a. Useful for teaching medical students
 - b. Epidemiological research
 - c. Public health planning

CARE AND MAINTENANCE OF RECORDS

Proper maintenance ensures accuracy and legal validity

1. *Storage -*
 - a. Keep in secure, fire proof areas
 - b. Maintain confidentiality
 - c. No unauthorised access
2. *Preservation -*
 - a. Water proof ink is preferred
 - b. No overwriting, use corrections properly
- c. Keep documents intake without tearing and staple damage
- d. Follow digital records security protocols (password, encryptions)
3. *Retention period -*
 - a. Medico legal records are kept for indefinite period as per state rules

- b. Other records are kept for minimum 3-10 years depending on hospital policy
- 4. *Chain of custody*
 - a. Label, seal, sign, date and document presenting over on evidence
 - b. Prevent tempering

OBJECTIVES OF MAINTAINING REPORTS

Medical and medico legal reports are prepared with the following goals -

1. Provide accurate, unbiased facts to legal authorities
2. Offer expert medical opinion in legal matters
3. Maintain clarity and completeness for court use
4. Document injuries, cause of death or findings scientifically
5. Assist in criminal investigation
6. Serve as permanent, legally admissible evidence
7. Protect both doctor and patient legally.

MEDICAL RECORDS

Medical records include all the documents related to the care of the patient.

Contents -

1. Patient identification
2. History and examination findings
3. Diagnostic result
4. Treatment given
5. Progress rate
6. Consent form
7. Summary/discharge rate
8. Medico-legal information

Characteristics -

1. Accurate records
2. Complete
3. Chronological
4. Confidential
5. Legible
6. Authenticated with doctors' signature and seal

DIFFERENCE BETWEEN RECORD AND REPORTS

records	reports
1. Continuous written documentation of patient care 2. Created at the time of events 3. Mainly for treatment, administration and reference 4. Maintain routinely in hospitals 5. E.g. case sheets, registers, lab charts	1. Normal statement summarising findings and opinion 2. Prepare later on request by authority 3. Mainly for legal or official purpose 4. Prepared only when needed for court/ police 5. E.g. injury report, postmortem report, age certificate

HOSPITAL

INTRODUCTION TO THE HOSPITAL

According to WHO (world health organisation), a hospital is an integral part of social and medical organisation, the function of which is to provide complete healthcare both preventive and curative to the population. It may or may not also provide services for ambulatory patients on an out-patient basis

THE MAIN OBJECTIVES OF THE HOSPITALS ARE

1. To prevent adequate care and treatment to its patient
2. Its principal products are medical, surgical, and nursing service to the patient and its central concern is the life and health of the patient
3. Additional objectives may include hospital's maintenance and survival, organisational stability and growth, financial solvency, medical and nursing education and research and various employ related objectives.

HISTORY OF THE HOSPITALS

1. *Ancient period* -
 - a. early care was provided in temples, monasteries and homes
 - b. ancient civilisations like Egypt, Greece, India, had healing centres
 - c. in India, charaka and Sushruta samhitas mention hospitals
2. *medieval period* -
 - a. hospitals were mainly run by religious institutions (Christian missionary, buddhist monasteries). Purpose was care, charity and nursing
3. *modern period* -
 - a. 18th -19th century - hospitals become more scientific with advances in anatomy, surgery and anesthesia etc.
 - b. Introduction of germ theory, sterilization and nursing system (Florence nightingale)
 - c. 20th century onwards - development of speciality hospitals, ICUs, diagnostic centre and advanced technology

CLASSIFICATION OF HOSPITALS

In the year 1988 the directory of hospitals in India classified hospitals into various types -

1. *Based on objectives*
 - a. **General hospitals** - it should have at least two medical doctors, more than one medical discipline, sufficient in-patient accommodation and should have active medical care. E.g. GMC & hospitals
 - b. **Rural hospitals** - these are similar to general hospitals but they are situated in rural areas. E.g. Azara rural hospital
 - c. **Specialised hospitals** - these hospitals are specialised in one particular discipline. E.g. Marwari maternity hospital
 - d. **Teaching cum research hospitals** - a hospital to which a college is attached for medical education. E.g. GMCH
2. *Based on administration, ownership or control or financial hospitals*

- a. **Govt./public** – govt. hospitals are run by central govt. or state govt. e.g. GMCH
- b. **Non-govt /private** – hospitals which are owned by an individuals or group of doctors or professionals or entrepreneurs e.g. nemcare hospital
- c. **Semi-govt hospital** – these hospitals are run by both govt. and private bodies on receiving funds from govt. e.g. Marwari maternity hospital
- d. **Voluntary agencies hospital** – these hospitals are run by not for profit of organisation . e.g. down town hospital
- e. **Corporate hospital** – these hospitals are run by public. E.g. apollo hospital
3. *Based on length of state*
 - a. **Short term or short stay hospital** – when the patient need is to stay less than 30 days for their medical care. E.g. govt hospitals
 - b. **Long term or long stay hospitals** – when the patient need to stay for more than 30 days. E.g. cancer hospital, mental hospital
4. *Based on bed capacity*
 - a. **Small hospitals** – which have up to 100 bedded capacity
 - b. **Medium hospital** – those with more than 100 less than 300 beds
 - c. **Large hospitals** – those with more than 300 beds capacity
5. *Based on type of care*
 - a. **Primary care** – these hospitals usually consists of general physician and does not have specialist. E.g. Dispur primary health centre, Dispur
 - b. **Secondary care** – these hospitals consists of specialist like dermatologist, neurologist. E.g. ESIC, ghy
 - c. **Tertiary hospitals** – advanced level of care for patients such as surgeries, transplantation, cancer management, neurological disorders are taken care by tertiary hospitals. E.g. GMCH
6. *Based on system of medicine*
 - a. **Allopathic hospitals** – hospitals that follows allopathic systems in which patients are treated with drugs and medicine. E.g. GMCH
 - b. **Ayurvedic hospitals** – hospitals that follow ayurvedic systems in which patients are treated with traditional systems of india. E.g. govt. ayurvedic hospitals, ghy
 - c. **Homeopathic hospitals** – hospitals that follow homeopathic systems in which patient are treated with drugs which stimulates healing process. E.g. SIN
 - d. **Unani hospital** – hospitals that follow unani system. In which patients are treated with herbal herbs and diet practices. E.g. Unani hospital, J&K
 - e. **Hospitals of other systems of medicine** – yoga & natural pathetic are other types of systems which help patient by drugless treatment like acupuncture. E.g. institute of yoga & anthropathy, ghy
7. *Depending on type of medical staff*
 - a. **Closed staff hospital** – physicians are held responsible for all medicine activities in the hospital including the diagnosis and treatment of the patient

- b. **Open staff hospitals** – this type of hospital permits other physician in the community to admit and treat patient to the hospital and treat them
- 8. *By teaching affiliations*
 - a. **Teaching hospitals** – e.g. GMCH
 - b. **Non-teaching hospitals** – e.g. civil hospitals
- 9. *As per WHO classification –*
 - a. **Regional hospital**
 - b. **Intermediate or district hospital**
 - c. **Rural hospital**

MEDICAL ETHICS

Medical ethics is the branch of ethics dealing with moral principles that govern the conduct of medical practitioner in their professional practice

PURPOSE OF MEDICAL ETHICS

1. To maintain dignity of the medical profession
2. To protect patient's right
3. To guide doctors in ethical decision making

PRINCIPLES OF MEDICAL ETHICS

There are four basic principle in medical ethics

1. Autonomy

- a. Respect patient's rights to make decisions
- b. Includes right to refuse treatment

2. Beneficence

- a. Act in the best interest of patient
- b. Promote patient's welfare

3. Non - maleficence

- a. Do not harm
- b. Avoid unnecessary injury or suffering

4. Justice

- a. Fair and equal treatment to all patients
- b. No discrimination based on caste, religion, gender, and wealth

5. Patient's right

- a. Respect, dignity and privacy
- b. Information about diagnosis, treatment and prognosis
- c. Inform consent before

procedures

- d. Confidentiality of medical records
- e. Refused treatment
- f. Safe and quality health care
- g. Access to medical reports

6. Confidentiality

Confidentiality means keeping patient's information private.

Exception required by law are

- a. Court order, police
- b. Consent given by patient
- c. Public interest
- d. Medical emergencies

7. Inform consent

It is voluntarily agreement made by patient after full disclosure to undergo a procedure

Essential elements are -

- a. Competent patient
- b. Voluntary decision
- c. Adequate information
- d. Underlying of risk, benefit & alternatives

8. Written consent

Consent should be written form

Types of written consent -

- a. Implied consent :
routine examination

- b. Express consent :
written / oral

- c. Proxy consent :
given by guardian

CODE OF CONDUCT (duties of a doctor)

Duties towards patient -

1. Provide competent care
2. Maintain confidentiality
3. Be honest and compassionate

Duties towards profession

1. Uphold dignity of profession
2. Avoid false advertising
3. Continue medical education

Duties towards colleagues

1. Mutual respect
2. No professional rivalry

MEDICAL ETIQUETTE

Etiquette refers to acceptable professional behaviour. Examples:

1. Polite communication with patients

2. Respecting colleagues
3. Proper dress and demeanour
4. Not criticising another doctor unfairly

PROFESSIONAL MISCONDUCT

Any act violating ethical rules or professional duties. Examples -

1. Negligence
2. Issuing false medical certificate
3. Advertising and misleading patient
4. Accepting commission
5. Breach of confidentiality
6. Unethical sex determination

PUNISHMENT

1. Warning
2. Suspension of registration
3. Removal from medical register

DIFFERENCE BETWEEN ETIQUETTE AND MISCONDUCT

Etiquette	Professional misconduct
1. Expected behaviour 2. Moral obligation 3. Not punishable	1. Unacceptable behaviour 2. Punishable offence 3. Legally & non - legally punishable

medical etiquette is the moral code guiding doctors to practice medicine with respect to patient's right, professional dignity and societal trust

DEPARTMENTS OF A HOSPITAL

The major departments of a hospital are as follows :

1. *PATIENT CARE DEPARTMENT***a. Medical department**

The medical department consists of various specialised departments such as cardiology, neurology, medicine and surgery. The overall administration and functioning of these departments are under control of the medical superintendent

b. Nursing department

The nursing department consists of nursing staff who manage nursing services and nursing education in the hospital. The overall administration of this department is controlled by the nursing superintendent.

c. Paramedical department

The paramedical department is responsible for analysis of clinical samples and helps in diagnosis of patients. It consists of paramedical staff mainly of laboratory technician.

d. Pharmacy department

The pharmacy department is responsible for the purchase, distribution, storage and maintenance of drugs in the hospital. This department consists of qualified pharmacist

e. Radiology department

The radiology department helps in radiographic examinations such as X-RAY, CT-SCAN, ultrasonography (USG) and MRI. This department requires experienced and qualified radiologists.

f. Diet and nutrition department

The diet and nutrition department is responsible for planning and providing nutritious and balanced diet for patients according to the doctor's prescription. It ensures proper nutrition to support recovery and overall health.

2. BUSINESS MANAGEMENT

- a. **Admission department** - the admission department takes care of admitting patients to the hospital. It ensures queries related to patient and provides information about hospitals services
- b. **Administrative department** - the administrative department manages the overall administration of the hospital. It is led by a group of administrative staff, managers and administrators who ensures smooth functioning of the hospital.
- c. **Purchase department** - the purchase department is responsible for purchasing supplies and equipment required for healthcare services in the hospital
- d. **Account department** - the accounts department handles the financial aspects of the hospital. This includes managing purchases, collecting money from patients for healthcare services provided and maintaining overall financial management of the hospital.
- e. **Medical records department** - the medical records department maintains the medical records of patients and helps the hospital in handling medico-legal cases.
- f. **Central sterile supply department (CSSD)** - the central sterile supply department supplies sterile items to the hospital. This department receives unsterile items, washes, sterilizes and circulates them for reuse.
- g. **Housekeeping department** - the housekeeping department is responsible for the cleanliness and hygiene of the hospital and helps

maintain a safe and healthy environment

- h. **Social service department** - the social service department acts as a link between the patient and the hospital. It assists patients and their families in social, emotional and financial matters
- i. **Maintenance department** - the maintenance department is responsible for the maintenance and repair of hospital buildings, equipment and utilities

RESPONSIBILITY OF A HOSPITAL

1. **Patient care service** - a hospital provides safe, effective, affordable and continuous health care services. It ensures proper diagnosis, treatment, rehabilitation and follow up care
2. **Patient safety and quality assurance** - the hospital maintains infection control measures, sanitation and safety protocol to ensure high quality patient care
3. **Administrative management** - administrative management ensures efficient planning, organizing, staffing, coordination and smooth functioning of all hospital departments
4. **Infrastructure and facility management** - it ensures the availability of equipment, beds, utilities, emergency services, and their proper maintenance.
5. **Human resource management** - it recruits, trains, schedules and monitors staff. It also ensures good communication, discipline, and professional behaviour among employees
6. **Medical record and information management** - it maintains accurate, confidential and up to date patient records and ensures proper information management
7. **Ethical, legal and policy compliance** - it protects patient's rights, privacy and confidentiality, the hospital follows medical ethics, hospital policies are legal regulations
8. **Financial management** - it ensures transparency, proper use of funds, and prevents financial misuse.
9. **Training, teaching and research** - the hospital supports academic programmes for medical, nursing, physiotherapy and paramedical students. It also promotes research and evidence-based practise.
10. **Community health and public responsibility** - the hospital conducts health camps, awareness programmes and outreach services to improve community health.
11. **Emergency and disaster preparedness** - the hospital maintains 24*7 emergency services. It prepares for disasters management and mass casualty management to handle emergencies effectively,

FUNCTIONS OF A HOSPITAL

1. **Curative function** - the hospital provides diagnosis, treatment, rehabilitation and emergency medical care
2. **Prevention of disease** - it promotes immunization and controls communicable and non - communicable diseases. Health promotion and

health education are also provided.

3. **Surveillance centre** - the hospital monitors reliable morbidity and mortality data of the area to support public health planning.
4. **Education and research** - the hospital provides basic education and training programmes for undergraduate and postgraduate doctors, nurses and paramedical staff
5. **Professional support** - the hospital provides medical and surgical support services through trained healthcare professionals
6. **Early detection of epidemics** - the hospital ensures early detection and reporting of epidemics and creates awareness among the community
7. **Primary health care programmes** - the hospital supports community health development and provides basic and continuous learning for healthcare workers, considering social and economic factors.